

**Universal Catastrophic Coverage with Health Savings Empowerment
(A Structural Replacement for the ACA's Premium-Subsidy Architecture)**

Ten-Page Policy Brief

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Status: Internal CBO-style model estimates; not an official Congressional Budget Office score.

1. The Problem: The ACA Expanded Coverage but Preserved the Wrong Financing Structure

The United States does not suffer from a shortage of health-care spending. It suffers from a misallocation of spending. Public programs, employer plans, and households continue to finance a system that routes both catastrophic medical events and routine care through comprehensive insurance. That structure weakens price discipline, increases administrative complexity, obscures routine-care prices, and ties federal subsidy growth to premium inflation rather than to household financial protection.

The Affordable Care Act expanded coverage and reduced some forms of uninsured exposure, but it did not correct this underlying architecture. ACA premium tax credits rise with benchmark premiums. Cost-sharing reductions and actuarial-value rules preserve comprehensive-plan design. Essential health benefit requirements maintain broad benefit structures. Exchange administration adds complexity without restoring meaningful price discovery. The result is a system that protects many households from immediate loss of coverage while leaving federal finances exposed to long-term comprehensive-plan inflation.

The central fiscal problem is not simply that federal subsidies exist. It is that the subsidy base is wrong. Current federal support subsidizes comprehensive insurance premiums that bundle routine benefits, provider pricing power, network design, administrative load, mandated benefits, and catastrophic risk into one product. When benchmark premiums rise, federal obligations rise. UCC/HSE replaces that structure with a financing model that separates catastrophic protection from routine-care payment.

The core policy judgment is simple: **insurance should be used primarily for large, unpredictable, financially destabilizing medical events. Routine and predictable care should be financed through transparent, portable, prefunded accounts, with targeted federal support for low-income households, children, and chronic-condition patients.**

2. The Proposal: Universal Catastrophic Coverage Plus Health Savings Empowerment

Universal Catastrophic Coverage with Health Savings Empowerment, or UCC/HSE, restructures American health financing into two layers.

The first layer is **Universal Catastrophic Coverage**. Eligible individuals receive standardized catastrophic protection against major medical expenses exceeding a defined catastrophic threshold. The insured layer covers high-acuity hospitalization, trauma, major surgery, intensive-care services, severe acute illness, transplant-related hospitalization, major oncology episodes, and other financially ruinous events. Catastrophic risk is pooled and federally stabilized.

The second layer is **Health Savings Empowerment**. Routine, predictable, preventive, pediatric, behavioral-health, and chronic-care needs are financed through portable Health Savings Empowerment accounts. HSE accounts receive household contributions, employer contributions, and formula-based federal deposits. Federal deposits vary by income, pediatric status, age, and certified chronic-condition need. Low-income households and medically vulnerable patients receive prefunded routine-care liquidity rather than being asked to absorb a deductible without support.

UCC/HSE is not a repeal-only framework. It is not a narrow HSA expansion. It is not a reduction in catastrophic protection. It is a replacement architecture built around:

Component	Function
<i>Standardized catastrophic coverage</i>	Protects against large, financially ruinous medical events
<i>Federal catastrophic credits</i>	Replace ACA-style premium subsidies tied to comprehensive benchmark plans
<i>HSE accounts</i>	Finance routine, predictable, pediatric, behavioral-health, and chronic-care needs
<i>Federal high-cost-claim reinsurance</i>	Stabilizes the catastrophic layer by absorbing extreme claims
<i>Employer-market reform</i>	Redirects employer participation away from tax-favored comprehensive premiums
<i>Medicaid refocusing</i>	Preserves Medicaid for LTSS, ABD, medically complex children, HCBS, and CHIP continuity
<i>Chronic-condition safeguards</i>	Provides additional routine-care liquidity for predictable disease-management needs
<i>Rural stabilization</i>	Protects access-sensitive rural providers during routine-care repricing
<i>Supplemental-coverage guardrails</i>	Prevents insurers and employers from rebuilding ACA-style comprehensive coverage below the threshold
<i>Fiscal guardrails</i>	Keeps federal obligations bounded, formula-based, and adjustable

The proposal applies primarily to the **non-Medicare acute-care financing architecture**: ACA exchange coverage, Medicaid expansion and other non-disabled adult acute-care pathways, employer-sponsored comprehensive insurance, and uninsured household exposure. Medicare remains outside the reform. Traditional Medicare, Medicare Advantage, Part D, Medigap, and Medicare eligibility are not altered. Medicaid is not abolished; it is refocused on the populations and services for which Medicaid is structurally best suited.

3. What Changes and What Does Not Change

UCC/HSE changes the financing architecture of the non-Medicare market, but it does not eliminate public protection.

What does not change:

Medicare remains unchanged. Traditional Medicare, Medicare Advantage, Part D, Medigap, and Medicare eligibility remain outside the UCC/HSE replacement population.

Medicaid remains for long-term services and supports, Aged/Blind/Disabled populations, medically complex children, home- and community-based care, and CHIP continuity.

Catastrophic medical events remain insured. Hospitalization, trauma, emergency surgery, ICU admission, major oncology episodes, severe acute illness, and other high-cost events remain protected through universal catastrophic coverage.

Children and chronic-condition patients receive explicit support. Pediatric routine care and certified chronic-condition management are not left to household exposure alone.

What changes:

Routine care moves away from comprehensive insurance. Primary care, standard labs, low-acuity imaging, routine specialist visits, generic medications, behavioral-health follow-up, and chronic monitoring increasingly move into HSE-financed direct payment and transparent routine-care markets.

Federal subsidies move away from comprehensive premiums. ACA premium tax credits, cost-sharing reductions, and exchange-based subsidy machinery are replaced by catastrophic credits, HSE deposits, and federal reinsurance.

Employer health financing is recomposed. Employers may continue contributing to workers' health financing, but the federal tax preference for comprehensive employer-sponsored insurance is phased down. Employer support shifts toward wages, HSE deposits, catastrophic-plan facilitation, and limited supplemental benefits.

Supplemental products are limited. UCC/HSE allows narrow supplemental coverage, but prevents federally supported wraparound products from recreating comprehensive first-dollar insurance below the catastrophic threshold.

4. Empirical Basis: Health Spending Is Concentrated

The UCC/HSE model rests on a basic empirical fact: health-care spending is highly concentrated. Most people incur modest annual medical costs, while a smaller share of the population accounts for a disproportionate share of total spending. That distribution supports a distinction between routine-care financing and catastrophic risk pooling.

The technical model uses MEPS HC-251 person-level expenditure data to reconstruct annual medical spending across the non-Medicare population. The model does not treat the deductible as the government’s cost unit. Instead, it estimates **expected excess spending above the catastrophic threshold**. This is a critical scoring distinction. If a person spends less than the threshold, that person contributes zero to catastrophic liability. If a person spends above the threshold, only spending above the threshold enters the catastrophic benchmark.

In the current model, the simplified MEPS HC-251 threshold calculation reports:

<i>MEPS-based metric</i>	Estimate
<i>Total non-Medicare medical spending</i>	~\$1.414 trillion
<i>Weighted non-Medicare population</i>	~266 million
<i>Non-Medicare persons with annual spending ≥ \$7,500</i>	40.73 million
<i>Share of non-Medicare population above threshold</i>	15.10%
<i>Weighted spending above \$7,500 threshold</i>	\$778.91 billion
<i>Share of total non-Medicare spending above threshold</i>	55.08%
<i>Weighted spending below threshold</i>	\$635.14 billion
<i>Share of total non-Medicare spending below threshold</i>	44.92%

This result is central to the proposal. Roughly 15 percent of the non-Medicare population accounts for a majority of spending above the modeled threshold. That is exactly the type of spending distribution for which catastrophic risk pooling is economically appropriate. At the same time, below-threshold spending remains large enough that routine-care financing must be carefully designed. UCC/HSE therefore pairs catastrophic protection with prefunded HSE deposits rather than leaving households exposed without liquidity.

5. Federal Fiscal Scoring: Transitional Cost, Long-Run Savings

The current estimates are internal CBO-style model estimates, not official CBO scores. The scoring framework uses current-law federal subsidy baselines, MEPS HC-251 catastrophic-spending reconstruction, CMS national spending projections, employer-exclusion tax expenditure estimates, and the UCC/HSE scoring workbook. The technical brief is explicit that the purpose is not to replace a formal CBO score, but to present a transparent fiscal architecture that can be reviewed, recalibrated, and stress-tested.

The model has three fiscal components:

First, it estimates gross federal outlays under UCC/HSE: catastrophic credits, federal HSE deposits, reinsurance, and transition or administrative costs.

Second, it estimates offsetting savings and revenues relative to current law: ACA subsidy replacement, Medicaid adult acute-care transition, employer-exclusion reform, administrative simplification, routine-care repricing, and related structural savings.

Third, it compares gross outlays with offsetting savings to estimate net federal impact over 10-, 20-, and 30-year horizons.

Current Modeled Fiscal Summary

<i>Fiscal measure</i>	<i>Modeled result</i>
<i>10-year net federal transition cost</i>	+\$763.09 billion
<i>20-year cumulative federal savings</i>	\$7.462 trillion
<i>30-year cumulative federal savings</i>	\$32.93 trillion
<i>10-year direct consumer savings</i>	\$2.203 trillion
<i>10-year Medicaid consumer savings</i>	\$127.92 billion
<i>10-year federal catastrophic credits</i>	\$6.013 trillion
<i>10-year federal HSE deposits</i>	\$5.283 trillion
<i>10-year federal reinsurance</i>	\$264.9 billion

The model costs more than current law during the first decade because it front-loads two forms of protection: catastrophic coverage and routine-care liquidity. The long-run fiscal improvement comes from replacing open-ended comprehensive-premium subsidies with bounded catastrophic credits, fixed HSE formulas, employer-exclusion reform, administrative simplification, routine-care repricing, and federal reinsurance.

UCC/HSE does not claim that consumer shopping alone solves American health care. Routine-care price transparency matters, but the fiscal engine is structural: the reform changes what federal subsidies are tied to. Instead of subsidizing comprehensive premium inflation, federal support is attached to catastrophic protection, formula-based routine-care deposits, and high-cost-claim stabilization.

6. Vulnerable Populations: The Model Is Not Ordinary High-Deductible Insurance

The strongest objection to catastrophic-first reform is that high deductibles can harm low-income and medically vulnerable patients. That objection is valid when catastrophic designs are built around exposure alone. UCC/HSE is structured differently.

The proposal separates the insurance product from financing beneath the deductible. The catastrophic plan remains uniform, but routine-care liquidity varies by income, age, pediatric status, and certified chronic-condition need. Low-income households receive prefunded HSE deposits. Children receive pediatric routine-care support. Certified chronic-condition patients receive additional HSE deposits, condition-based support, and routine-care liquidity for predictable disease-management needs.

This is the central equity claim of UCC/HSE: vulnerable populations are protected through targeted financing beneath a universal catastrophic backstop, not by recreating ACA-style comprehensive insurance tiers.

Medicare. Medicare remains unchanged. No Medicare savings are used to finance the reform.

Medicaid. Medicaid is refocused, not abolished. Medicaid remains responsible for LTSS, ABD populations, medically complex children, HCBS, and CHIP continuity. Non-disabled adult acute-care populations transition into catastrophic coverage and HSE-financed routine care.

Children. Pediatric routine care is financed through age-adjusted HSE deposits and coordinated with Medicaid and CHIP continuity rules.

Chronic-condition patients. Chronic-condition support is delivered through enhanced HSE deposits and condition-based routine-care financing beneath the deductible. The catastrophic plan remains uniform.

Emergency-department backflow. UCC/HSE includes an ER non-backflow guardrail. The model recognizes that if routine-care liquidity fails, low-income patients may defer care and return to emergency departments. The design attempts to prevent that by prefunding routine-care accounts before bills arise.

7. Provider Impact and Rural Stabilization

UCC/HSE also answers a practical delivery-system question: what happens to hospitals and providers when routine care moves away from comprehensive insurance and toward transparent HSE-financed markets?

The provider impact is best understood as **revenue re-composition**, not a uniform provider cut. Catastrophic and high-acuity care remains insured. Routine and shoppable outpatient services face greater price discipline. Providers dependent on opaque facility-fee billing and high-margin routine outpatient claims face pressure. Independent clinics, ambulatory surgery centers, imaging centers, laboratories, direct primary care practices, and efficient specialist groups may benefit from routine-care migration toward transparent markets.

The current provider-impact model uses FY2024 CMS HCRIS hospital cost-report data. It evaluates routine-care repricing exposure, uncompensated-care and bad-debt offsets, transition stabilization, rural access protection, and behavioral-health institutional adjustments. The model is static and first-order; it is not a hospital-closure forecast and does not claim every provider is held harmless.

Provider Impact Summary

<i>Provider-impact measure</i>	Modeled result
<i>Provider backbone</i>	912 providers
<i>Modeled providers</i>	886 providers
<i>Total net patient revenue</i>	~\$264.9 billion
<i>Total outpatient/routine exposure</i>	~\$135.2 billion
<i>Outpatient repricing pressure</i>	-\$13.52 billion
<i>UC/liquidity offset</i>	+\$1.91 billion
<i>Bounded stabilization support</i>	+\$0.30 billion
<i>Net provider impact, Base/Base</i>	-\$11.30 billion
<i>Net provider impact after preferred access protection</i>	~-\$11.08 billion

Rural providers require separate treatment. Rural hospitals face low volume, fixed standby capacity, thin labor markets, long transfer distances, and limited nearby substitutes.

UCC/HSE therefore includes a rural access-capacity payment equal to 2 percent of net patient revenue, capped at \$5 million per qualifying rural provider, plus a temporary

implementation-period floor preventing qualifying rural providers from falling below -1.5 percent of net patient revenue.

Rural Access Summary

<i>Rural measure</i>	Before protection	After preferred access protection
<i>Rural / CAH / IHS net impact</i>	-\$0.341 billion	-\$0.123 billion
<i>Net impact as share of rural NPR</i>	-3.1%	-1.1%
<i>Rural providers below -5% NPR</i>	10	0
<i>Rural providers below -10% NPR</i>	Not retained under preferred policy	0

This preserves the core economic logic of the reform while addressing access-sensitive providers. Rural stabilization is not used to restore opaque outpatient pricing. It is provider-side, formula-based, capped, and tied to access capacity.

8. Comparison to ACA and Medicare for All

UCC/HSE occupies a different structural category from both the ACA and Medicare for All.

The ACA subsidizes comprehensive private insurance. Its main fiscal risk is that federal subsidies rise with benchmark premiums. Medicare for All federalizes comprehensive financing. Its main fiscal challenge is that a much larger share of health financing moves onto the federal budget, with affordability depending heavily on provider-payment compression, administrative savings, drug pricing, and new federal revenue. UCC/HSE instead limits the federal role to catastrophic protection, HSE liquidity, and federal reinsurance.

ACA vs Medicare for all vs UCC/HSE

<i>Feature</i>	ACA	Medicare for All	UCC/HSE
<i>Core model</i>	Subsidized comprehensive private insurance	Federalized comprehensive financing	Universal catastrophic coverage plus HSE-financed routine care
<i>Federal exposure</i>	Benchmark premiums and subsidies	Broad federal responsibility for most medical spending	Catastrophic credits, HSE deposits, reinsurance
<i>Routine care</i>	Embedded in comprehensive insurance	Federally financed	HSE accounts, direct payment, employer deposits, targeted support
<i>Catastrophic care</i>	Covered through comprehensive plans	Federally financed	Universally insured through standardized catastrophic layer
<i>Employer role</i>	Preserved through tax exclusion and group plans	Largely displaced or heavily reduced	Recomposed toward wages, HSE deposits, catastrophic facilitation, limited supplemental benefits
<i>Medicaid</i>	Expanded for low-income adults; retained for traditional populations	Often absorbed or restructured	Refocused on LTSS, ABD, medically complex children, HCBS, CHIP
<i>Medicare</i>	Preserved	Becomes platform for broader federalization	Preserved unchanged
<i>Main fiscal risk</i>	Premium-linked subsidy growth	Federalization of comprehensive spending	Transition cost and parameter calibration

The policy choice is not simply between markets and government. The more precise question is which parts of health financing require public pooling and which parts require transparent routine-care markets. UCC/HSE answers: catastrophic risk should be pooled; routine care should be financed transparently with liquidity support and safeguards.

9. Implementation and Guardrails

UCC/HSE is designed as a phased replacement architecture, not an overnight conversion.

Year 1: Congress authorizes UCC catastrophic plans, establishes the national reinsurance pool, creates HSE accounts, authorizes federal catastrophic credits and deposits, and directs CMS/Treasury implementation.

Year 2: UCC catastrophic plans become broadly available. Existing ACA exchanges remain operational for transition administration but close to new enrollment.

Year 3: ACA premium tax credits, cost-sharing reductions, and exchange subsidy mechanisms phase out for the replacement population. UCC catastrophic credits and HSE deposits become the primary federal financing mechanisms.

Years 4–5: Medicaid non-disabled adult acute-care populations complete transition into UCC/HSE. Employer-market transition continues through staged tax-exclusion phase-down and employer HSE contribution pathways.

The framework also includes fiscal and benefit-design guardrails. Catastrophic credits are tied to a defined catastrophic benchmark, not comprehensive premiums. HSE deposits are formula-based. Reinsurance parameters have defined attachment points. Supplemental coverage is restricted to prevent benefit creep. Annual reporting and automatic adjustment mechanisms are designed to prevent UCC/HSE from becoming a new open-ended entitlement.

10. Bottom Line for Congressional Offices

UCC/HSE is a universal but non-single-payer health-financing architecture. It preserves catastrophic financial protection, leaves Medicare unchanged, refocuses Medicaid on its core institutional responsibilities, and replaces ACA premium-linked subsidies with bounded catastrophic credits, HSE deposits, and federal reinsurance.

The proposal's central claim is not that every assumption is final. The central claim is that the ACA's subsidy architecture is structurally inflation-sensitive, and that a more durable federal framework should separate catastrophic risk pooling from routine-care financing. The current technical model provides a transparent first-order scoring framework using MEPS HC-251, CMS projections, CBO/JCT baselines, and FY2024 HCRIS provider-impact analysis. It is intended for review, recalibration, and formal stress-testing.

UCC/HSE can be summarized in one sentence:

Insure catastrophic risk universally, finance routine care transparently through prefunded HSE accounts, protect vulnerable populations explicitly, stabilize high-cost claims federally, and stop tying federal health obligations to comprehensive-plan premium inflation.